DECISION MEMO | DOAC FIELD STRATEGY

Target the gap, not the volume.

THE ANSWER

Keep the same 60 reps and cost. Re-target their calls on a 50/50 blend of modelled opportunity and volume, redraw the territories, and hold headcount until a two-quarter pilot measures what a call is worth.

WHY WE BELIEVE IT

In held-out 2024 the blended list won 1.19x the share gains of today's volume list and kept 91% of its absolute growth (clears the 90% floor in 86% of re-draws). Flagged prescribers grew 1.40x as fast as propensity-matched peers. A blend weight tuned on the earlier year failed out of time; 50/50 did not, because it was never tuned.

THE MAP

Territories drawn alphabetically by state carry a 106x workload imbalance and 20% contiguity. The redraw: 3.6x, 100% contiguous, travel 70% shorter.

WHY NOT HIRE (OR CUT)

Public data show no measurable call response. Today's 60th rep pays only if a year of calls captures 1.72% of a prescriber's headroom. Hiring pays only if calls beat that by 1.48x, after hiring, ramp-up and severance costs; the 2026 Medicare negotiated price raises the bar a further ~1.6x.

$4.2M/yr	$4.5M	90%	$0
re-targeting, zero cost	pilot vs deciding blind	pilot power, six months	new headcount before read-out

THE PILOT

Randomly hold out 20% of the new list (no calls) for two quarters; score both arms on brand-share change with a prior-year adjustment; read at month six. Power 90% on share points (19% if scored on fills). Below 0.5x break-even: shrink or redeploy. Above 2x: reopen hiring with a measured number.

RISKS

Reps keep calling old accounts (CRM nudges, on-list incentive); hold-out contamination (hidden in CRM, monthly audit); Medicare-only data, ~17 months old (refresh each CMS release).

DECISIONS NEEDED

1. Approve the blended call list. 2. Approve the territory redraw. 3. Freeze headcount at 60 for two quarters and fund the 20% hold-out pilot.

Every number is read from the pipeline output (data/manifest.json). Real CMS Part D and Open Payments data, 2022-2024; client and brand illustrative.